Challenge Stigma / Find Peace

FrontlineFolks | Guide #31

Sex and Avoidance After Difficult Experiences

A Field Guide for Frontline Folks and Their Families

Field notes for the pattern in this chapter.

By Michael Jenkins, MS, MSW, LCSW
Never Forgotten Services, PLLC

Disclaimer

This guide is for general education only.

It is not:

- Therapy
- Medical care
- A crisis service
- Individualized advice

Reading this guide does not create a therapist–client relationship with the author or with Never Forgotten Services, PLLC.

If you are in crisis or thinking about harming yourself or someone else, call 988 in the United States, go to your nearest emergency room, or contact your local emergency services.

If you want mental health treatment, please contact a licensed provider in your state or call the number on the back of your insurance card.

Table of Contents

1. What Is Happening
2. Why It Escalates
3. What To Do This Week
4. For Partners and Team
5. When To Get More Help
6. 7-Day Field Drill
7. Worksheets
    - Worksheet 1: Worksheet: Sex and Avoidance After Difficult Experiences
    - Worksheet 2: Worksheet: Sex and Avoidance After Difficult Experiences - Partner/Team Check-In

1. What Is Happening

After traumatic exposure, sex becomes dangerous. Your partner tries to touch you and something in your brain says 'threat.' You might freeze, become numb, or desperately want them to stop. This isn't about not loving your partner—it's about your nervousness system associating sex (intimacy, vulnerability, physical closeness) with the trauma. Maybe something your partner does reminds you of the trauma (a particular touch, sound, or position). Maybe the vulnerability of sex feels impossible when you're in hypervigilance mode. You avoid sex entirely, or you go through motions but dissociate the whole time.

Common trauma-related sex avoidance patterns

- Desire is present, but avoidance is automatic: You want sex intellectually, but your body refuses when it's time.
- Triggered by specific acts or positions: Certain touches, angles, or contexts directly trigger trauma memories.
- Dissociation during sex: You're physically present but mentally checking out, watching yourself from a distance.
- Inability to relax into vulnerability: Your nervous system says 'no one can touch you now', including your partner.
- Startle response: Your partner's touch—even gentle touch—causes you to spike in activation.
- Persistent shame: Feeling broken, fearing you'll never be able to have sex again, guilt about your partner's rejection.
- Avoidance escalating: You've said no so many times, initiating feels impossible now.

> **This is not your failure:**
> Trauma-related avoidance is your nervous system protecting you. Your brain thinks sex = danger. That's not character, that's neurobiology.

2. Why It Escalates

Trauma creates an association between vulnerability/physical closeness and danger. Your nervous system learned 'let your guard down = violated.' Sex requires dropping your guard. So your body says no. Add hypervigilance (you're scanning for threats constantly) and the lack of safety in your own body, and sex becomes a context where you feel exposed and undefended. If the trauma was interpersonal violence (assault, domestic violence), vulnerability is even more laden with danger.

How trauma avoidance escalates

- Triggered memories during sex: A specific touch, position, or moment activates the trauma itself.
- Dissociation as protective strategy: You check out mentally to survive physically, then feel numb/disconnected.
- Partner pressure and resentment: Your partner wants connection, you're saying no, they feel rejected, you feel pressured.
- Avoidance becomes habitual: You've said no so many times, initiating feels impossible and partner stops trying.
- Shame and isolation: You stop talking about it. The rupture in your sexual relationship creates rupture in connection.
- Unresolved trauma compounding: Until you process the trauma, the avoidance stays locked in.

3. What To Do This Week

You can rebuild sexual connection, but only after you address the trauma response. This requires trauma work first, then gradual rebuilding together.

1. Get trauma-focused treatment: EMDR, CPT, PE, or somatic work all help reduce the trauma activation during sex.
2. Take sex completely off the table temporarily: Zero expectation, zero pressure. Physical affection is allowed (non-sexual touch), but sex isn't happening.
3. Rebuild parasympathetic activation: Your nervous system needs to learn 'physical closeness is safe again.' This takes time and repetition.
4. Identify safe touch: Work with your partner to find one way of being touched that feels safe (gentle hand-holding, back scratch, etc). Start there.
5. Practice grounding during touch: If you start to dissociate, practice grounding techniques: name 5 things you see, feel the temperature of their hand, etc.
6. Communicate triggers clearly: 'That position reminds me of the trauma' or 'I need to start slower.' Your partner can't help if you don't name it.

7. Gradual, guided return to sex: Only when trauma response has decreased, work with partner (and possibly a sex therapist) to gradually rebuild sexual intimacy.
8. Couple's trauma therapy: Processing trauma together (in appropriate therapy) can rebuild safety in the relationship.

> Field rule: set the bar low enough to do it even on your worst day.

4. For Partners and Team

Stress communication should be clear, brief, and repeatable. Predictable routines protect relationships.

- Use concrete language: 'That touch activated trauma for me. This isn't about you.'
- Agree on complete pressure release: Sex is off-table until you're ready. Rebuilding happens gradually, never by force.
- Debrief without blame: 'I felt triggered. I need to work on this. Can you help me feel safe?'
- Consent, pacing, and emotional safety are part of intimacy: This is not 'push through.' This is 'rebuild safety first, sex second.'

---

5. When To Get More Help

Self-guided tools are a start. Escalate care when symptoms persist, spread, or begin harming safety and function.

- Trauma-related avoidance has lasted 6+ months and isn't improving with self-care.
- Your partner is becoming resentful or considering ending the relationship.
- You're having intrusive trauma memories during or after intimate moments.
- Avoidance is spreading: You're avoiding touching altogether, showering, certain clothing.
- Sex therapy or trauma-informed couples care (trauma-focused and sex-informed) can rebuild safety and connection.

6. 7-Day Field Drill

Use the worksheet every day for one week. Track trigger, body signal, and reset action. Keep what works and remove what does not.

> **Daily drill:**
> 1) Name the trigger.
> 2) Name the first body signal.
> 3) Run one reset action.
> 4) Log response quality.

Worksheet 1: Worksheet: Sex and Avoidance After Difficult Experiences

Use this to track the pattern in this chapter and build a practical response you can repeat under load.

Where did this pattern show up this week?

List specific moments at work, at home, and in transition.

What did it cost you when it took over?

Name impact on body, mood, decisions, and relationships.

What two actions will you run for the next 7 days?

Choose actions small enough to use on hard days.

1. Trigger:

What activated the pattern?

2. Early body signal:

What did your body do first?

3. Reset action used:

What fast action lowered intensity?

4. Better next response:

What exact behavior will you use next time?

> Track this daily for one week, then tighten the plan around what actually worked.

Worksheet 2: Worksheet: Sex and Avoidance After Difficult Experiences - Partner/Team Check-In

Use this page for one short practical check-in each week.

What do others notice first when this pattern is active?

Capture observable behavior, not labels.

What support helps most and least?

List what helps and what escalates the issue.

What agreement will you both practice this week?

One clear action from each side.

1. Signal phrase:

What phrase means pause and reset?

2. Time-to-return commitment:

When will re-engagement happen?

3. Repair step:

What specific repair happens after a rupture?

> Keep check-ins short, specific, and repeatable.

Closing

You've seen and carried more than most people will ever know.

Blow ups and shutdowns are not proof that you're broken or that your relationship is doomed.

They are signs that your nervous system has been running in survival mode for a long time.

The fact that you picked up this guide and read it matters.

You are choosing to be intentional about how you show up for the people who matter to you.

That choice is a lot more powerful than having flawless days.

About the Author

Michael Jenkins, MS, MSW, LCSW

Founder, Never Forgotten Services, PLLC

Michael is a U.S. Army combat veteran and licensed clinical social worker who specializes in working with military, veterans, and first responders—our frontline folks. After leaving the military, he spent years fighting his own demons and trying to figure out why he could hold it together on the job but feel completely wrecked at home. The turning point was realizing he wasn't the only one living that split life—and that nobody should have to feel that alone, confused, or broken by what the job asks of them.

Since then, his work has focused on trauma, the impact of high-stress jobs on home life, and helping families make sense of what the job brings home. He uses plain language, zero shame, and a "you're not crazy, this actually makes sense" approach to help frontline folks and their families understand what's happening in their nervous systems and learn how to move through the chaos together.